

Management Case Study

Children's Healthcare of Atlanta

Strategic Analysis, Change Management, and Recommendations

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Overview

Children's Healthcare of Atlanta (CHOA) is Georgia's only freestanding pediatric health system and the state's only system fully dedicated to the care of children and adolescents. The nonprofit organization operates three hospitals, Arthur M. Blank Hospital, Scottish Rite Hospital, and Hughes Spalding Hospital, together with the Marcus Autism Center, the Center for Advanced Pediatrics, the Zalik Behavioral and Mental Health Center, urgent care centers, and a broad network of neighborhood locations. CHOA reports more than 1.2 million patient visits annually, more than 1,200 pediatric specialists, over 14,500 team members, and more than 60 specialties and programs (Children's Healthcare of Atlanta [CHOA], 2026a, 2026b).

The opening of Arthur M. Blank Hospital in September 2024 marked a substantial expansion of the system's clinical capacity. The 19-story, two-million-square-foot facility includes 446 licensed beds, 69 emergency examination rooms, and 22 operating rooms, along with expanded capacity in oncology, hematology, and cardiac services. From a health systems perspective, this investment gives CHOA meaningful room to meet future demand, but it also raises the stakes for workforce planning, care coordination, service line management, and disciplined capital stewardship (CHOA, 2026c).

CHOA is entering a new strategic period. Patrick Frias, MD, became chief executive officer on August 31, 2026, assuming responsibility for the organization's emerging More Than Possible strategy. The strategy centers on placing trusted pediatric services within 30 minutes of every metro Atlanta family, closing pediatric healthcare deserts, expanding rural partnerships, strengthening mental and behavioral health services, and sustaining investment in clinical innovation (CHOA, 2026d).

This case study applies core public health and health administration frameworks, including SWOT analysis, Kotter's change model, and a structure, process, outcome lens drawn from Donabedian's quality framework, to assess how CHOA can manage continued growth while preserving its pediatric mission, organizational culture, financial strength, and quality of care. The analysis relies on public information rather than internal performance data. Identified weaknesses therefore represent structural management exposures and areas warranting continued attention, not confirmed internal findings.

System Snapshot

System Snapshot	Publicly Reported Indicator
Clinical reach	More than 1.2 million patient visits annually across three hospitals and multiple neighborhood locations
Workforce	14,500+ team members and more than 1,200 pediatric specialists
New capacity	Arthur M. Blank Hospital: 446 licensed beds, 116 more than the former Egleston Hospital
Research	\$59 million in 2025 NIH funding; \$112.9 million in total extramural funding; 168 new clinical studies
Financial position	\$2.61 billion in 2024 net patient service revenue and \$10.73 billion in total net assets

Strategic Position and SWOT Analysis

STRENGTHS • Clear pediatric mission and statewide brand • Three hospital network with major new capacity • More than 1.2 million annual visits and 60+ specialties • Strong Emory academic partnership and national research standing • Substantial financial assets and positive operating income • Established culture standards centered on respect, accountability, collaboration, and excellence	WEAKNESSES / MANAGEMENT EXPOSURES • High fixed cost and coordination demands following major capital expansion • 34.7% of 2024 patient revenue came from Medicaid and other government programs • Persistent access gaps across metro and rural Georgia • Dependence on a large, specialized pediatric workforce • Simultaneous leadership transition, site growth, access expansion, and clinical innovation increase implementation complexity
OPPORTUNITIES • Implement the More Than Possible access strategy • Expand hub and spoke specialty care, telehealth, and Project ECHO • Build the rural pediatric workforce through Mercer and KidsABC • Scale behavioral health through the \$550 million endowment and the Epic learning health system architecture • Translate research, data, and cell and gene therapy advances into clinical impact • Use new facilities to redesign patient flow and family experience	THREATS • National shortages of nurses, pediatricians, and other specialists • Rising wages, supply costs, and hospital operating expenses • Medicaid reimbursement and uncompensated care pressure • Growing behavioral health demand that may exceed available capacity • Execution risk if rapid expansion outpaces workforce, access, and process standardization • Competition for specialized talent, research funding, and patient referrals

Strategic interpretation: CHOA's central management question is not whether to diversify beyond pediatrics; its pediatric specialization is a defining strength and a genuine source of competitive advantage. The real question is how to extend that expertise to more Georgia children while protecting clinical quality, workforce capacity, organizational culture, and financial discipline.

SWOT Interpretation

Strengths

CHOA's strongest advantage is a highly differentiated mission supported by scale. Its statewide reputation, specialized workforce, advanced facilities, and family-centered care model create an identity that general hospital systems cannot easily reproduce. Arthur M. Blank Hospital substantially increased capacity and introduced advanced infrastructure, including the state's only Level I pediatric trauma center. CHOA's research ecosystem is another major asset: in partnership with Emory University School of Medicine, its pediatric research program ranked third nationally for NIH funding in 2025 and reported \$112.9 million in total extramural research funding (CHOA, 2026e).

CHOA also holds a strong financial base. Audited statements reported \$2.61 billion in net patient service revenue, \$2.91 billion in total operating revenue and support, \$335.5 million in operating income, and \$10.73 billion in net

assets for 2024. Philanthropic contributions were meaningful but accounted for only \$135.4 million of operating revenue and support, indicating that the system is not primarily financed by donations (CHOA, 2025).

Weaknesses and Management Exposures

Public information does not point to major internal weaknesses, but it does surface structural exposures that management must actively control. The first is operational complexity. A three-hospital system with more than 14,500 employees, numerous specialty sites, a new flagship hospital, and a broad research enterprise requires consistent processes, referral pathways, data governance, and cross-site communication. The second is the fixed cost burden associated with advanced pediatric care and major capital assets; CHOA reported \$2.57 billion in 2024 operating expenses and more than \$1.12 billion in long-term debt.

The system also carries meaningful public payer exposure. Medicaid and other government programs accounted for 34.7% of 2024 patient service revenue, and CHOA reported \$412.4 million in total community benefit costs for 2023, including unreimbursed Medicaid and charity care. These figures do not signal instability, but they do create sensitivity to reimbursement policy and to the underlying cost of caring for underserved children (CHOA, 2025).

Opportunities

The new leadership period creates an opening to align facilities, workforce, community partnerships, and digital care around measurable access goals. The More Than Possible strategy aims to place pediatric care within 30 minutes of metro Atlanta families and to reduce pediatric healthcare deserts. In rural Georgia, CHOA serves more than 25,000 children each year, has committed a \$200 million sustainable fund, and is partnering with Mercer University and the Georgia Rural Health Innovation Center to expand local pediatric capacity. Thirty-four future physicians had committed to the rural scholarship pipeline as of August 2026 (CHOA, 2026d, 2026f).

Behavioral health and research offer additional platforms for growth. CHOA has established a \$550 million behavioral health endowment, serves more than 8,000 new behavioral health patients annually, and is developing an Epic-based learning health system. Its research infrastructure supports clinical trials, data-driven improvement, and emerging cell and gene therapies (CHOA, 2026g).

Threats

External threats include workforce shortages, cost growth, reimbursement pressure, and escalating demand. HRSA projects a national shortage of 9,320 pediatricians and broader shortages across most physician specialties by 2038, with the greatest gaps concentrated in nonmetropolitan communities (Health Resources and Services Administration [HRSA], 2025). These conditions are likely to increase recruitment costs and could slow statewide access expansion. Hospital labor and supply expenses are also rising nationally, placing continued pressure on operating margins even for financially strong organizations such as CHOA.

Strategic Management and Access Issues

CHOA's current management challenge is to convert major capital investment into reliable access and measurable outcomes for Georgia's children. The organization has already added hospital capacity and publicly committed to expanding its geographic reach. The next phase requires operational integration: ensuring that patients enter the right level of care, that referrals move efficiently across the network, that specialty capacity is visible in real time, and that community partners can draw on CHOA expertise without sending every child to Atlanta.

Network Coordination

The growth of neighborhood sites, rural partnerships, hospital campuses, behavioral health services, and specialty programs introduces genuine coordination risk. Management should define shared referral standards, access metrics, service line ownership, and escalation pathways. A system-wide dashboard could monitor referral completion, appointment lead time, avoidable emergency utilization, geographic access, transfer time, and patient and family experience, functioning as a population-level early warning system for access strain.

Financial Stewardship

CHOA's financial statements show substantial strength, but they also illustrate why disciplined management remains essential. Net patient service revenue is the primary operating source, Medicaid and other government payers represent more than one third of that revenue, and workforce, benefits, purchased services, and supplies account for much of operating expense. Strategic expansion should be evaluated through combined access, quality, equity, workforce, and financial measures rather than through patient volume alone.

Rural and Community Access

CHOA cares for children from all 159 Georgia counties, including all 120 rural counties. Fifty seven percent of its rural patients are enrolled in Medicaid or PeachCare or are otherwise unable to cover the full cost of care. This underscores the need for care models that keep appropriate services close to home. Telehealth, Project ECHO, physician-to-physician consultation, standardized transfer protocols, and regional workforce development can extend pediatric expertise without requiring CHOA to build a full specialty facility in every community.

Behavioral Health Capacity

CHOA's behavioral health investments are already substantial, so the opportunity is not simply to create another program but to integrate prevention, primary care, crisis triage, outpatient treatment, and outcomes monitoring into a single continuum. CHOA reports that comprehensive outpatient services have contributed to a nearly 40% reduction in repeat emergency visits for behavioral crises among participating patients. Management can build on this result by standardizing referral criteria and extending consultation support to pediatricians, schools, and community partners.

Management Priorities and Sample Performance Measures

Management Priority	Sample Performance Measures
Access	Days to next available appointment; percentage of metro families within 30 minutes; rural referral completion
Capacity	Bed and operating room utilization; specialty waitlists; transfer acceptance time
Workforce	Vacancy, turnover, time to fill, engagement, workload, and internal mobility
Equity	Access and outcomes by geography, payer, race and ethnicity, language, and service line

Management Priority	Sample Performance Measures
Financial stewardship	Cost per encounter, payer performance, contribution margin, and community benefit impact

Organizational Culture and Workforce Engagement

CHOA publicly describes its employee promise as People First, Children Always. Its Green Standard emphasizes respect, positive interactions, accountability, integrity, excellence, support, and collaboration. This values framework aligns well with pediatric care, where outcomes depend on multidisciplinary teamwork and on trust among clinicians, staff, patients, and families (CHOA, 2026b).

The transition to a new chief executive and the launch of an ambitious statewide strategy together create a critical culture management period. Growth can strengthen mission engagement, but it can also generate change fatigue when teams must adapt simultaneously to new facilities, workflows, technology, service locations, and performance expectations. Leaders should translate More Than Possible into practical meaning for each role and invite employees to identify barriers before fully designing new initiatives, consistent with participatory approaches to organizational change.

Culture Risks During Expansion

- Different sites or service lines may develop inconsistent practices as the network expands.
- Operational pressure may make employees see the strategy as added workload rather than improved access to care.
- Rapid recruitment can dilute shared expectations if onboarding and leader development do not scale with hiring.
- Centralized decisions may overlook the lived experience of frontline teams and rural or community partners.

Culture Building Actions

CHOA should use the Green Standard as an operating framework rather than only an employee-facing message. Respect can be measured through patient, family, and employee experience data. Accountability can be reinforced through clear ownership and transparent performance reviews. Excellence can be tied directly to clinical outcomes and access. Collaboration can be embedded in cross-functional improvement teams that include frontline staff, administrative leaders, physicians, families, and community partners.

Leadership rounding, structured listening sessions, stay interviews, team huddles, and rapid cycle feedback would help surface implementation problems early. Managers should also receive training in psychological safety, conflict management, workload planning, and change communication. This approach would support retention while preserving the mission-centered identity that differentiates CHOA within the broader health care market.

Culture recommendation: Connect every major access initiative to one Green Standard behavior, one accountable leader, one frontline feedback mechanism, and one measurable patient or workforce outcome.

Change Management and Strategy Implementation

Kotter's eight-step change model is well suited to CHOA's More Than Possible strategy because the work crosses facilities, professions, and community partnerships. The model emphasizes urgency, coalition building, vision, communication, barrier removal, short-term wins, acceleration, and institutionalization (Kotter, 2012).

Step	Application to CHOA
1. Create urgency	Use geographic access, workforce, behavioral health, and appointment data to show where Georgia children still face barriers.
2. Build a coalition	Include hospital and ambulatory leaders, clinicians, operations, finance, IT, families, community partners, Mercer, Emory, and rural providers.
3. Form the vision	Translate the 30-minute access goal and statewide mission into specific service line and regional objectives.
4. Enlist participation	Communicate what will change, why it matters, and how each team contributes; create structured channels for staff input.
5. Remove barriers	Simplify referrals, align scheduling and transfer protocols, address workforce gaps, and improve data visibility across sites.
6. Generate wins	Pilot access improvements in one metro healthcare desert and one rural region, then publish early access and outcome results.
7. Sustain acceleration	Use pilot results to refine workflows and expand the model across additional communities and specialties.
8. Anchor the change	Incorporate access, equity, collaboration, and improvement measures into leadership reviews, onboarding, budgeting, and recognition.

The model should be implemented through phased pilots rather than a single, simultaneous system-wide rollout, consistent with a RE-AIM-oriented approach to scale up (reach, effectiveness, adoption, implementation, maintenance). A pilot allows CHOA to test staffing assumptions, referral logic, data collection, family communication, and community partnerships before scaling. It also gives employees visible, credible evidence that the strategy can improve care rather than simply increase workload.

The principal risk is that change becomes leadership-driven but operationally disconnected. To avoid this, frontline teams should participate directly in design decisions and in the review of performance data. Leaders should report both positive results and unresolved barriers, so that trust is maintained throughout implementation rather than eroded by selective communication.

Recommendations

The following recommendations are ranked by urgency and organized using a structure, process, and outcome logic: each recommendation strengthens organizational structure or process capacity in service of measurable access and equity outcomes. Difficulty is scored on a scale from 1 (easiest) to 5 (most difficult).

Recommendation	Urgency	Diff	Advantages	Limitations / Risks
1. Establish a system-wide access and capacity command structure	Immediate	3/5	Creates shared definitions, ownership, and visibility for wait times, referrals, transfers, capacity, equity, and the 30-minute access goal.	Requires data integration, governance, and agreement across service lines; poorly designed metrics could reward volume rather than value.
2. Expand the rural hub and spoke pediatric network	High	4/5	Keeps appropriate care closer to home, strengthens local clinicians, reduces travel burden, and builds on the \$200 million rural commitment and the KidsABC partnership.	Specialty coverage, broadband, licensure, workflow, and local capacity may limit scale; some children will still require transfer.
3. Build an integrated workforce and retention strategy	High	4/5	Supports new capacity, protects culture, reduces vacancies, and aligns recruitment, training, internal mobility, well-being, and rural workforce pipelines.	Requires sustained long term investment; national pediatric and nursing shortages cannot be solved by one health system alone.
4. Scale the behavioral health learning system	High	4/5	Connects prevention, triage, treatment, Epic data, research, and outcomes; can extend already documented reductions in repeat crisis visits.	Data governance, workforce constraints, privacy, integration with community services, and sustainable reimbursement add complexity.

Recommended Sequence

CHOA should begin with the access and capacity command structure, because it establishes the data infrastructure, accountability, and decision process needed for the other recommendations to succeed. The organization can then run two visible pilots: one focused on a metro Atlanta access gap and one focused on a rural hub and spoke partnership. Workforce planning and behavioral health integration should be embedded within both pilots rather than treated as separate, parallel initiatives.

Within the first 90 days, leadership should define outcome measures, select pilot regions, identify executive and operational owners, and conduct frontline listening sessions. During months four through nine, CHOA should implement the pilots and review performance monthly. At the close of the first year, leaders should decide which components to standardize, revise, or discontinue before pursuing broader expansion.

Overall management priority: Convert a broad growth strategy into a disciplined portfolio of measurable access improvements while protecting clinical quality, employee capacity, and financial sustainability.

Conclusion

Children's Healthcare of Atlanta enters its next strategic period from a position of considerable strength. Its pediatric specialization, statewide reputation, advanced facilities, research partnerships, workforce, and financial resources provide a strong foundation for continued growth. The core management challenge is to coordinate these assets so that expansion produces timely, equitable, and sustainable access rather than additional operational complexity.

The More Than Possible strategy appropriately centers on healthcare deserts, geographic access, rural partnerships, behavioral health, and clinical innovation. Successful implementation will require shared performance measures, cross-functional governance, workforce development, frontline participation, and phased, evidence-informed change. By anchoring growth decisions to its Green Standard and using pilot results to guide expansion, CHOA can strengthen access to pediatric care while preserving the mission and culture that make the organization distinctive within Georgia's health care landscape.

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